INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Carboplatin and vinorelbine · lung cancer

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00614
Version 4

Four 21 day cycles are planned, unless treatment stops or changes earlier. Day 1: carboplatin plus vinorelbine. Day 8: vinorelbine only. Your prescription specifies vinorelbine by drip OR capsules; these are alternatives, not extra doses.

Treatment stages

01 **Four 21 day cycles**
Days 1 and 8

02 **Review after treatment**
Next plan agreed

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact oncology immediately for fever, shaking chills, sudden illness, breathing difficulty, chest pain, bleeding, uncontrolled vomiting or severe constipation with tummy pain. Report new numbness or weakness before the next dose. Tell staff immediately about infusion pain or swelling.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

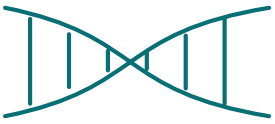
Your medicines

This chemotherapy aims to control lung cancer and symptoms when cisplatin is unsuitable. The team reviews benefit, side effects and your priorities.

VINCA ALKALOID CHEMOTHERAPY

Vinorelbine

Prescribed intravenous (IV) or oral route



Disrupts microtubules needed for cell division

Vinorelbine disrupts the cell structures that help divide chromosomes. This can slow cancer growth and also affect blood cells, nerves and the bowel.

Binds cell-division structures → Microtubules cannot function normally → Cell division is disrupted

What to watch for

Report fever, constipation, numbness or pain at the infusion site promptly.

CHEMOTHERAPY

Carboplatin

Platinum chemotherapy



Damages deoxyribonucleic acid (the cell’s genetic material) (DNA)

Carboplatin forms links in DNA that hinder copying and repair. Healthy tissues can also be harmed. Its dose depends on target exposure (AUC) and measured or estimated kidney function; AUC is not a fixed number of milligrams. There are two planned doses in this regimen.

DNA → Platinum links → Copying disrupted

What to watch for

Report fever, bleeding, nausea, hearing changes or numbness. Tell the nurse immediately about possible allergy during the infusion.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding the evidence

Carboplatin with vinorelbine has been studied in advanced lung cancer.

Historical phase II evidence used alternating IV/oral vinorelbine, not every route variant, and cannot quantify comparative benefit.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your overall plan

Treatment stages

01 **Four 21 day cycles**
Days 1 and 8

02 **Review after treatment**
Next plan agreed

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

DAYS 1 AND 8

Four 21 day cycles

1 2 3 4

Four 21 day cycles are planned, unless treatment stops or changes earlier. Day 1: carboplatin plus vinorelbine. Day 8: vinorelbine only. Your prescription specifies vinorelbine by drip OR capsules; these are alternatives, not extra doses.

NEXT PLAN AGREED

Review after treatment

Review

Four cycles take about 12 weeks without delays. Your team agrees response scans and what happens next. Surgery or radiotherapy is not automatically included.

Four cycles may change for benefit, toxicity or patient preference.

BASELINE → TREATMENT → FOLLOW-UP

My tests and checks

Required safety checks are distinct from individual response imaging and follow-up. No routine tumour-marker interval is specified.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00614 v4 · official regimen

THE WHOLE TREATMENT JOURNEY

Carboplatin and vinorelbine · lung cancer: treatment and checks

Advanced/recurrent non-small-cell lung cancer (NSCLC) when cisplatin is unsuitable

Four 21 day cycles are planned, unless treatment stops or changes earlier. Day 1: carboplatin plus vinorelbine. Day 8: vinorelbine only. Your prescription specifies vinorelbine by drip OR capsules; these are alternatives, not extra doses.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before treatment	Treatment days	Between doses	Next cycle	After treatment
Treatment NCCP 00614	Confirm both drugs and route	Day 1both; Day 8vinorelbine	No other routine dose	Repeat up to 4 cycles	Next plan agreed
Blood tests NCCP 00614	full blood count (FBC), kidneys/liver, GFR	Pre-dose FBC; renal input	Extra tests if indicated	FBC/renal/ liver each cycle	Follow abnormalities
Nerves / bowels NCCP 00614	Baseline nerve review	Review symptoms	Report constipation/ numbness	Neuropathy each cycle	Review persistent effects
Scans / other care NCCP 00614	Disease stage and goal	No fixed scan day	As team directs	Response review	Individual next plan

What each check is for

1 Prepare

Before first dose

This chemotherapy aims to control lung cancer and symptoms when cisplatin is unsuitable. The team reviews benefit, side effects and your priorities.

Ask / act: Confirm aim, prescription, tests and contacts.

2

Review

Before each dose

Check treatment remains appropriate

Ask / act: Report symptoms and confirm blood results before dosing.

3 Next plan

At agreed review points

Assess benefit and recovery

Ask / act: Confirm scan and follow-up dates with the team.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Confirm the prescribed route

Vinorelbine by drip and vinorelbine capsules are alternatives. Ask the team to mark your route.

STEP 2

Day 1 and Day 8

Day 1 includes carboplatin; Day 8 contains vinorelbine only. Arrange blood tests and clearance for each dose.

STEP 3

Capsules only when prescribed

Keep in the original packaging and follow pharmacy storage/handling instructions. Do not repeat a vomited dose or make up a missed dose.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Carboplatin + vinorelbine	 **Day 1** Day 1 only; after safety checks.
Vinorelbine only	 **Day 8** Day 8 only; after safety checks.
Symptom awareness	 **Days 1–21** Symptoms may occur on any day; this is not a personal risk curve.

Timing varies: Tiredness, bowel changes and infection can occur between visits. Blood tests and your symptoms guide dose decisions; there is no guaranteed safe day.

Changes to the plan: The team may delay, omit or reduce doses for safety. Do not add a missed dose yourself.

> **Why these bands stay broad:** Relative treatment days, without personal dates. Symptoms are not confined to a fixed day. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00614 v4 · official regimen ↗

> **There are no guaranteed "safe days".** Contact oncology immediately for fever, shaking chills, sudden illness, breathing difficulty, chest pain, bleeding, uncontrolled vomiting or severe constipation with tummy pain. Report new numbness or weakness before the next dose. Tell staff immediately about infusion pain or swelling.

Between-treatment notes

Day 1: Carboplatin + vinorelbine

Both medicines after safety checks; confirm prescribed vinorelbine route. Your team confirms whether this dose can proceed.

Day 2–7: At home

No planned chemotherapy on these days. Use only the support medicines prescribed for today. Call using your contact plan if unwell; do not wait for a treatment day.

Day 8: Vinorelbine only

No carboplatin today. Vinorelbine by the prescribed route after safety checks. Your team confirms whether this dose can proceed.

Day 9–21: At home

Support around the medicines

1 Different support on Day 1 and Day 8

Day 1 carboplatin needs a stronger anti-sickness plan. Day 8 depends on whether vinorelbine is by drip or capsules. Use the written prescription for that day and route; do not repeat the entire Day 1course automatically.

2 Bowel plan

Vinorelbine and some anti-sickness medicines can cause constipation. Follow the prescribed bowel plan and call for severe tummy pain, swelling or vomiting.

3 If capsules are prescribed

Swallow whole with water and some food. Do not chew or open capsules. Do not take an extra dose after vomiting; contact the team for missed-dose advice. Return damaged capsules to pharmacy.

4 Check medicine interactions

The team should check all medicines, including anti-sickness treatment. Do not start new tablets, supplements or vaccines without advice.

Questions worth asking

Am I having both oral and IV vinorelbine?

No. Your written plan must identify the prescribed route for each dose. Do not treat capsule instructions as additional treatment.

Why is Day 8different?

Only vinorelbine is planned. Blood results, symptoms and the route determine whether dosing and the support plan are appropriate.

What if treatment changes?

Ask your team to update the cycle record, prescription and contact plan. A stamp is not permission for the next dose.

Try explaining the plan back: Explain back your treatment days, checks and whom to contact when unwell.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · carboplatin plus prescribed vinorelbine

Active medicines: Carboplatin + Vinorelbine

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1; 80 mg orally Days 2–3	Before systemic anticancer therapy (SACT) on Day 1, then daily on Days 2–3
Ondansetron	16 mg orally	Day 1 Adjust for QT/individual risk.
Antiemetic dexamethasone reference	12 mg orally Day 1; 8 mg orally daily Days 2–4	Dedicated carboplatin AUC ≥4 NCIS V 8 row Reference course only: reconcile the aprepitant interaction and any other steroids into one clear prescription.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS row does not specify an end day or PRN qualifier; prescriber/pharmacy must specify duration and suitability.

Additional phase-specific support

- Confirm prescribed vinorelbine route and review CYP3A4 inhibition by aprepitant; vinca toxicity may increase.
- Use one reconciled antiemetic prescription; do not add a second oral-vinorelbine5HT3agent to Day 1ondansetron.
- Use a constipation-prevention plan and assess bowel symptoms.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Day 8 · intravenous (IV) vinorelbine only

Active medicines: Vinorelbine

Emetogenic risk: Minimal IV risk

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Individual antiemetic plan differs by route; list routine and rescue medicines separately.
- Discuss constipation prevention, hydration/diet where appropriate and prescribed laxatives; regular opioid use increases the need for a bowel plan.
- No routine steroid premedication, granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), antibacterial, antiviral or Pneumocystis jirovecii pneumonia (PJP) prophylaxis is mandated by these NCCP monotherapy documents. Consider additional prevention based on individual risk/local policy; do not create a generic automatic prescription.
- Gastric protection is a consideration for high gastrointestinal-bleed risk, not a universal drug.

- Confirm strong CYP3A4 inhibitors/inducers, P-gp modulators, phenytoin, itraconazole, warfarin and live vaccines. Yellow fever vaccine is contraindicated. Avoid grapefruit per NCCP; do not self-start supplements.
- No routine prophylaxis required by NCCP minimal-risk pathway.
- Prescriber selects an as-needed medicine according to symptoms, constipation, QT/interactions and local policy. Reassess before future doses if nausea/vomiting occurs.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Day 8 · oral vinorelbine only if prescribed

Active medicines: Vinorelbine

Emetogenic risk: Moderate-to-high oral risk

Medicine	Dose / route	When
One 5-HT3 antagonist · choose a single option	Ondansetron 8–16 mg total daily OR granisetron 1–2 mg total daily	Around the weekly dose; the prescriber must state the number of subsequent days NCCP Table 5 oral pathway. Do not combine both agents or assume indefinite daily treatment. Review constipation, QT risk and interacting medicines.

Additional phase-specific support

- Individual antiemetic plan differs by route; list routine and rescue medicines separately.
- Discuss constipation prevention, hydration/diet where appropriate and prescribed laxatives; regular opioid use increases the need for a bowel plan.
- No routine steroid premedication, granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), antibacterial, antiviral or Pneumocystis jirovecii pneumonia (PJP) prophylaxis is mandated by these NCCP monotherapy documents. Consider additional prevention based on individual risk/local policy; do not create a generic automatic prescription.
- Gastric protection is a consideration for high gastrointestinal-bleed risk, not a universal drug.
- Confirm strong CYP3A4 inhibitors/inducers, P-gp modulators, phenytoin, itraconazole, warfarin and live vaccines. Yellow fever vaccine is contraindicated. Avoid grapefruit per NCCP; do not self-start supplements.
- Table 5 says continue daily; for once-weekly vinorelbine the prescriber must state the number of days. Do not automatically turn this into indefinite daily treatment or import an intravenous (IV) moderate steroid/NK1 schedule.
- A separate locally approved as-needed plan; check constipation and QT/interactions. Avoid duplicate 5-HT3 agents. Ondansetron can worsen constipation.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Nerves

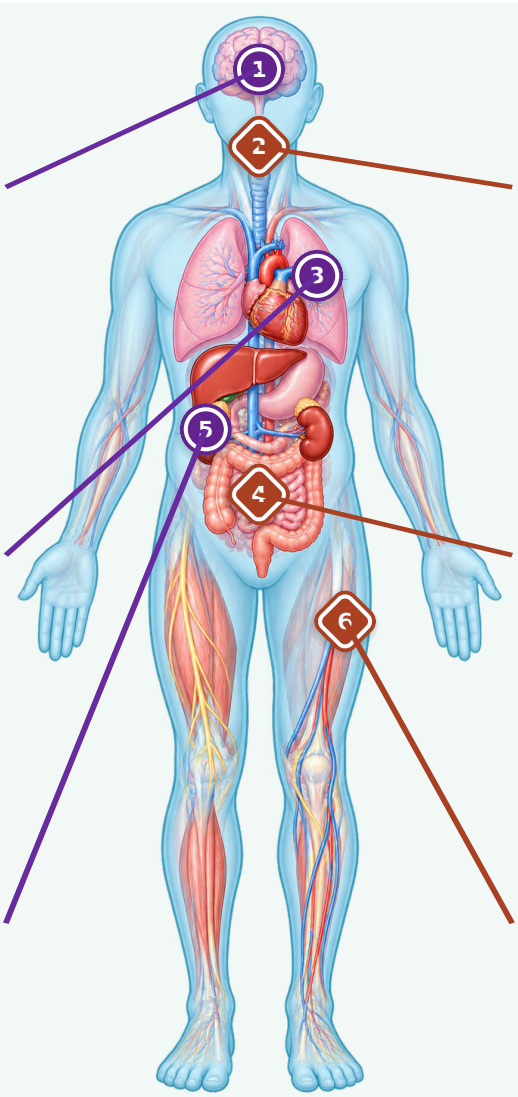
Vinorelbine

Report new numbness, tingling or weakness before the next dose.

2 Mouth

Both medicines

Report sores or pain that limits food or drink.



3 Breathing / allergy

Both medicines

Breathing difficulty or swelling needs urgent help.

4 Bowels / sickness

Both medicines

Severe constipation, pain, swelling or vomiting needs urgent advice.

5 Kidneys / dose checks

Carboplatin

Kidney tests guide dosing; report much less urine.

6 Blood cells / IV site

Both medicines

Call for fever or unusual bleeding; tell staff about infusion pain.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact oncology immediately for fever, shaking chills, sudden illness, breathing difficulty, chest pain, bleeding, uncontrolled vomiting or severe constipation with tummy pain. Report new numbness or weakness before the next dose. Tell staff immediately about infusion pain or swelling.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00614 v4 · official regimen**
   https://healthservice.hse.ie/documents/6884/614_V4_Carboplatin_Vinorelbine.pdf
2. **Navelbine intravenous (IV) product information**
   https://www.medicines.org.uk/emc/product/3912/smpc
3. **NCCP antiemetic guidance v6 · minimal IV pathway**
   https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf
4. **O'Brien et al.2004 carboplatin with alternating IV/oral vinorelbine**
   https://pubmed.ncbi.nlm.nih.gov/15151949/
5. **NCCP Antiemetic Medicines for inclusion in NCIS (Medical Oncology) · v8, 10 February 2025**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf